

GLOBAL HEALTHCARE: PHARMACEUTICALS

The Cardiology Renaissance: Post CARDIO-TTTransform, two top-of-mind catalysts remain for 3Q

Following the surprise failure of AZN/IONS' CARDIO-TTTransform, a highly-watched event for the ATTR-cardiomyopathy field, and subsequent volatility (AZN -10%, IONS -33%, ALNY -11%, BBIO +6% from the July 8 close, before the announcement), we look to two remaining key cardiovascular disease (CVD) catalysts likely in 3Q that could result in significant stock reactions and inform the opportunity for a number of novel approaches:

1. NVS/IONS' HORIZON trial of pelacarsen to read through to the Lp(a) class.
Investor expectations appear low given the aforementioned delays, Lp(a) as an unproven target, and NVS' commentary - per our statistical analysis, significance would be achievable in HORIZON's overall population and the high-risk subgroup on a 12% and 14% risk reduction, respectively. Nonetheless, we believe a trend in efficacy would provide positive readthrough for the field, and if HORIZON is successful, we expect commercial uptake to remain debated in the context of Lp(a) testing rates/payor dynamics, noting the emerging longer-acting or oral options (e.g., AMGN with data in 2027/28, LLY, among others).
2. NVO' ZEUS study of ziltivekimab (IL-6), which could validate that inflammation reduction as measured through lowering of high sensitivity C-reactive protein (hsCRP), translates to a reduction to CV risk reduction in ASCVD - a 15%-20% benefit would read favorably to BIOA's oral NLRP3 inhibitor with proof-of-concept data in YE.

We recognize there could be heightened caution into these events as they are perceived by investors as higher-risk than CARDIO-TTTransform. Within, we frame the set-up, including benchmarks for success, stock moves in the illustrative bull/bear scenarios - we calculate +2%/-3% and +7%/-5% for NVS and NVO, respectively - alongside options commentary, potential sympathy reactions from affected companies (e.g., IONS, AMGN, NAMS, BIOA), and broader CV catalysts in 2H26/2027.

Broadly, we remain positive on AMGN, ALNY and MRK as names best positioned to experience upside from large CV product cycles.

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Framing two top-of-mind cardiology catalysts

Recall, AZN and IONS announced that the Phase 3 CARDIO-TTTransform study (n= 1,432) of antisense oligonucleotide Wainua in ATTR-cardiomyopathy (CM) missed its primary endpoint of the composite outcome of cardiovascular (CV) mortality and recurrent CV clinical events assessed up to week 140 - while there was a nominally significant hazard ratio of 0.71 in the monotherapy arm, there was no evidence of benefit in combination with stabilizer therapy (e.g., PFE's tafamidis), and tafamidis-treated patients performed better-than-expected in the milder population.

The update was a negative surprise, as success on the primary endpoint was expected per our investor discussions and the key debate centered on whether the stabilizer combination group - 57% of patients with an additional 24% drop-in rate - would achieve significance. We await full results at the European Society of Cardiology (ESC) Congress in August.

After the CARDIO-TTTransform news, significant stock moves transpired for AZN (-10%), IONS (-33%), and affected companies ALNY (-11%) and BBIO (+6%). On the forward, we look to two other Phase 3 cardiovascular catalysts this quarter that could result in volatility for the subject companies as well as wide-reaching effects for the field: (1) NVS/IONS' HORIZON trial of Lp(a)-targeting pelacarsen; and (2) NVO's ZEUS assessing ziltivekimab (IL-6). Below, we provide the set-up into the catalysts, including benchmarks for success, readthrough to other affected companies, our implied up/down moves, and options commentary.

Exhibit 1: Key cardiology catalysts in 2026 and 2027

Company	Program	Modality	Class/Indication	Status	Event	Timing
NVS/IONS	pelacarsen	ASO	Lp(a)	Ph3	HORIZON	Mid-26
AZN/IONS	Wainua	ASO	TTR	Ph3	CARDIO-TTTransform detailed data at ESC	August
ARWR	plozasiran	siRNA	SHTG	Ph3	SHASTA-3/4	3Q
ARWR	ARO-DIMER-PA	siRNA	Mixed Hyperlipidemia	Ph1/2	Topline data	3Q
NVO	ziltivekimab	mAb	IL-6	Ph3	ZEUS	3Q
CRSP	CTX611	siRNA	Factor XI - TKA	Ph2	Top-line clinical data	2H
CRSP	CTX310	CRISPR	ANGPTL3 - SHTG/HeFH/HoFH	Ph1b	Clinical data	2H
CRSP	CTX321	CRISPR	Lp(a)	Preclinical	Program update	2026
ROIV	mosliciguat	sGC activator	PH-ILD	Ph2	PHocus trial data	2H26
NTLA	nex-z	CRISPR	TTR (PN)	Ph3	MAGNITUDE-2 enrollment completion	2H26
BIOA	BGE-102	Small molecule	ASCVD	Ph 2	Proof-of-concept data in obese patients with CV inflammation	YE26
BMJ/JNJ	milvexian	Small molecule	Factor Xla	Ph3	Ph3 readout in SSP and IF	YE26
MRK	enlincitide	Macrocytic peptides	LDL-C	Ph3 completed	Potential approval and launch	YE26- early 27
REGN	REGN7508	Antibody	Factor XI	Ph 3	Ph3 data in post-total knee replacement VTE vs. apixaban and enoxaparin	1Q27
NAMS	obicetrapib	Small molecule	CETP	Ph3	PREVAIL CVOT interim analysis	1Q27
REGN	REGN7508/9933	Antibody	Factor XI	Ph 2	Ph2 data vs. apixaban in atrial fibrillation	2Q27
AZN	balcinrenone + Farxiga	Small molecule	HF with CKD	Ph 3	BalanceD-HF	2027
AZN	laroprovstat	Small molecule	LDL-C	Ph 3	AZURE-HeFH and AZURE-LDL	2027
REGN	REGN7508	Antibody	Factor XI	Ph 3	Ph3 data in post-total knee replacement VTE vs. aspirin	2027
AMGN	olpasiran	siRNA	Lp(a)	Ph3	OCEAN(a)	2027/2028

Source: Company data, Visible Alpha Consensus Data, Goldman Sachs Global Investment Research

NVS/IONS: HORIZON - pelacarsen (Lp(a)) in cardiovascular disease

- **Background/Market Opportunity.** We see Lp(a)-targeting drugs as a potentially transformative, representing perhaps the last known significant cardiovascular target that has not yet been drugged despite being a risk factor for atherosclerotic cardiovascular disease (ASCVD) that impacts ~20% of the general population (~1.4 billion people globally, 60mn+ people in the US) - see our deep dive [here](#). Emerging Lp(a) directed therapies (see below) have reduced Lp(a) by up to ~80-90%+ in prior trials, albeit the potential causal relationship of lowering Lp(a) to improve CV outcomes is awaiting validation per topline data from NVS/IONS' HORIZON trial evaluating pelacarsen likely in 3Q (per NVS' guidance of an early 2H release). For pelacarsen, we model for \$2bn peak sales at a 60% PoS in our NVS model. We note key debates into HORIZON'S data have included:
 - The translation of Lp(a) lowering to CV risk reduction;
 - Whether greater Lp(a) reduction drives superior CV outcomes, which would benefit AMGN;
 - The commercial outlook and readthrough to other studies if the primary endpoint is achieved only in the high-risk subgroup OF horizon and not the overall;
 - The lack of benefit on high-sensitivity C-reactive protein, an inflammatory marker; and
 - AMGN's and NVS' trial design and baseline characteristics, particularly the isolation of Lp(a) via the optimal treatment of other risk factors (e.g., LDL-C), which is logical from a regulatory/payor perspective but likely impacted event rates.
- **Catalyst outlook.** The Ph3 HORIZON trial of monthly subcutaneous antisense oligonucleotide (ASO) pelacarsen vs. placebo is the first large-scale (n=8,323) pivotal study evaluating the effect of lowering Lp(a) on cardiovascular risk reduction (on expanded MACE; cardiovascular death, non-fatal MI, non-fatal stroke and urgent coronary re-vascularization requiring hospitalization) - thus representing a key catalyst for the field. NVS is testing the primary endpoint in the overall population (Lp(a) ≥ 70 mg/dL) and the high-risk subgroup (Lp(a) ≥ 90 mg/dL), where the events-driven study has >90% power to detect a hazard ratio (HR) of approximately 0.80 and 0.75, respectively.

Our analysis of the trial statistics suggests that the ≥ 70 mg/dL could achieve significance with a 12% benefit and the ≥ 90 mg/dL significance could be hit with a 14% benefit, noting management anticipates achieving statistical significance in the 13-15% range and believes that while such a range might be considered modest for LDL lowering, it could be meaningful for Lp(a). That said, uptake is a key area of debate, including Lp(a) testing rates and emerging competition with longer-acting and oral options - NVS is also investigating its next-generation, long acting DII235, which could move to Ph3 if proof-of-concept is achieved with pelacarsen in HORIZON.
- **Options commentary.** Assuming that the HARBOR trial for del-desiran and the REMODEL trials for remibrutinib in MS are expected to come in late September/

early October (base don clinicaltrials.gov primary completions), the options market (Sept expiry) is pricing in a +/-9.2% move for Novartis into HORIZON.

■ **Affected companies.**

- **NOVN/NVS.** Our illustrative analysis implies +2%/-3% potential stock reaction on a positive/ negative result. However, we also note that at the current multiple (c. 16x vs. sector average c. 14x 2027E EPS) we believe this already largely reflects success for all three upcoming key read outs, implying there could be greater downside risk, particularly if all three catalysts are not perfect, in our view (see [here](#)). Regarding IONS, who receives milestones and royalties on pelacarsen, flexing our model's 60% probability of success to 0% in the bear case implies at least 10% downside, while increasing the probability of success to 100% and increasing the peak revenue opportunity (e.g., for context, applying a factor of 1.5x on the royalty stream vs. our base case of peak unadjusted royalties of ~\$500mn) on the bull case of a highly positive trial implies 10-20% upside potential from our PT of \$68.
- **NAMS.** While the impact to NAMS is nuanced, we see potential for upside convexity to NAMS shares on a positive pelacarsen outcome in Novartis' HORIZON, given the Lp(a) lowering effect seen with NAMS obicetrapib. NAMS has previously presented pooled Ph3 data across three obicetrapib trials showing a median pbo-adjusted reduction of Lp(a) of up to 45% over 12 weeks ([link](#)) which has shown to be greater than that of PCSK9 agents (15-30%; [link](#)), however not as robust as focused Lp(a) lowering agents (80-95%; [link](#)). A positive result from Novartis' HORIZON trial would be viewed as further validation of obicetrapib's Lp(a) lowering causality as residual risk reduction on top of LDL-lowering. This would hence be supportive of NAMS management's hypothesis of obicetrapib's multi-mechanism benefits and serve to partially de-risk the company's critical PREVAIL CVOT trial (interim analysis expected near YE 2026, with readout early 2027) — in a company analysis interrogating the 21% MACE benefit seen in the Ph3 BROADWAY trial, NAMS estimated that obicetrapib's Lp(a) reduction potentially provides ~5% of that MACE reduction. NAMS believes that a negative HORIZON outcome would not significantly impact obicetrapib's prospects, as the residual contribution from LDL particles, diabetes-related effects, and other factors would likely be larger than estimated. In our DCF, holding all else constant and flexing the probability of success for obicetrapib up/down by 5% (we currently assume 75% in our model) implies approximately +/- 7% change to our DCF valuation. Illustratively, in a negative HORIZON outcome adjusting our model POS down 5%, implies -7% to our DCF valuation, in a positive outcome adjusting our model POS up 10% implies +14% to our DCF valuation, and in a mixed outcome our model would likely be unchanged or marginally adjusted dependent on the details.
- **AMGN/ARWR.** AMGN's quarterly siRNA olpasiran is likely more potent on Lp(a) reduction vs. pelacarsen per prior data; however, whether more potent reduction drives incremental benefit may depend on the baseline Lp(a) level. Primary completion of the Ph3 is expected by management in 2028, but

ultimately is dependent on event accrual rate. We do not include olpasiran in our AMGN model. With regard to stock performance on the back of a data update, our illustrative analysis implies the potential for 5-10% upside on Scenario 1 (as Street de-risks a potential blockbuster opportunity - e.g., assuming ~\$3bn peak sales and a 4x peak sales multiple - with multiple expansion representing additional upside), 3-5% upside in Scenario 2 (similar to Scenario 1, but less de-risking), and Scenario 3 (as Street removes and value, plus modest multiple compression).

- **LLY.** LLY has two programs in development, siRNA lepodisiran, with estimated (per clinical trials.gov) primary completion in March 2029 for the Ph3 ACCLAIM-Lp(a) study, and daily oral pill muvalaplin, with estimated (per clinical trials.gov) primary completion in March 2031 for the Ph3 MOVE-Lp(a) trial. We do not model these assets separately in our LLY model, rather recognize in the Other Ph II/III bucket.
- **CRSP.** CRSP is developing an in vivo gene editing asset, CTX321, designed to provide a one-time durable reduction in Lp(a). CTX321 is currently in IND/CTA-enabling studies, and incorporates an optimized guide RNA that delivered approximately two-fold greater potency in preclinical models over first-generation asset CTX320. We model for non-risk-adjusted global peak sales of \$1.3bn in 2040. Per our model, increasing the POS to 50% (from 15%) for CRSP's LPA program on positive readthrough from HORIZON suggests shares could trade up 5%, and we see potential for additional upside on a non-fundamental basis. Should HORIZON not meet statistical significance but demonstrate a trend, this could still provide positive readthrough to CRSP given the potential for superior LP(a) reductions via the gene editing approach - in this scenario, increasing the POS to 30% (from 15%) suggests ~2.2% upside potential. In the event HORIZON fails, decreasing the POS to 0% for CRSP's LPA program suggests shares could trade down ~2.2%, with additional downside potential on a non-fundamental basis.
- **MRK/Hengrui.** MRK licensed HRS-5346, an investigational oral small molecule Lp(a) inhibitor currently in Ph2 studies in China. We do not include this asset in our MRK model.
- **RPRX:** RPRX is entitled to an upward tiered mid-single-digit royalty from NVS's pelacarsen, as well as a low-double digit tiered royalty for AMGN/ARWR's olpasiran through 2041. We model a peak royalty of \$73mn for pelacarsen and ~\$440mn for olpasiran.
- **SLN.** SLN's siRNA zerlasiran that targets Lp(a) has reported Ph2 data, albeit the company continues to seek out a partnership before initiating a Ph3 CVOT trial. We model zerlasiran achieving ~\$897mn worldwide non-risk-adjusted peak sales in 2042.

Exhibit 2: Illustrative scenarios and potential stock moves for HORIZON

	ITT Benefit	NVS	IONS	AMGN	CRSP	NAMS
Scenario 1	Stat Sig / Meaningful	+2%+	+20%+	+5-10%	+5-10%	+14%
Scenario 2	No Stats but Signal	0-2%	+5-10%	+3-5%	Flat to +5%	~Flat (detail dep)
Scenario 3	Failure	-3%+	-10%+	-3-5%	-2-5%	-7%

Source: Goldman Sachs Global Investment Research

NVO: ZEUS - ziltivekimab (IL-6) in cardiovascular disease, chronic kidney disease and systemic inflammation

- **Background/ Market Opportunity.** Ziltivekimab is a humanized mAb to IL-6, which in turn is often a key factor in inflammatory diseases. The primary readout from the Phase 3 ZEUS trial of ziltivekimab in ASCVD is expected by management in 3Q26.
- **Catalyst outlook.** ZEUS is a Phase 3, randomized, quadruple-masked 48 month study of ziltivekimab vs placebo (1:1 randomisation) in 6,200 patients with ASCVD (atherosclerotic cardiovascular disease). Given there are no previous studies of ziltivekimab in cardiovascular indications, it is difficult to set an efficacy bar for success. However, in our experience, the industry standard for a cardiovascular medication to be deemed useful is showing a 15-20% efficacy benefit. In the ZEUS trial, this would translate to a 15-20% increase in the time to first occurrence of 3-point Major Adverse Cardiovascular Event (MACE). However, based on our calculations, we expect that a 12% MACE benefit should be enough to show statistical significance in the trial.
- We currently model ziltivekimab across all indications at a 40% PoS, with launch in 2028E and rising to peak risk-adjusted US sales of c.DKK 10.5bn/\$1.6bn in 2037E. Adjusting the PoS to 100% would increase our peak sales estimate to c.DKK 26.2bn/\$4bn, and imply c.7% upside potential to our DCF valuation; 0% PoS would imply c.5% downside risk to our DCF valuation.
- **Options commentary.** With the trial expected to come in 3Q'26 (Sept expiry), the options market is pricing in a +/- 14.3% move for the ZEUS trial for Novo shares.
- **Affected companies.**
 - **Novo:** As noted above, our illustrative analysis implies +7%/-5% impact on the shares, all else equal, on success/ failure of the ZEUS trial. In addition, and perhaps more importantly, a successful ZEUS trial could give Novo an anchor asset for its obesity/ diabetes/ CV adjacent portfolio, which could be a first step towards diversifying the portfolio/ reducing some of the concentration risk that comes with the long term semaglutide patent expirations.
 - **BIOA:** BIOA is advancing its oral, small molecule NLRP3 inhibitor BGE-102 in cardiovascular risk, following positive Ph1 MAD data which demonstrated rapid (within 2-3 weeks) and competitive (86%) reduction in hsCRP, a marker of CV inflammation, in patients with obesity and baseline inflammation. IL-6 is downstream to NLRP3 in the inflammasome pathway, and we expect 15-20% efficacy benefit on 3-point MACE to read favorably BIOA's approach in CV risk, further validating hsCRP as a surrogate for MACE post NVS' Ph3 CANTOS data for canakinumab (IL-1 β) which demonstrated compelling MACE benefit in a post-hoc analysis of patients who achieved hsCRP normalization (<2mg/L). BIOA is running a Ph2 proof-of-concept study for BGE-102 in obese patients with elevated CV risk, assessing % reduction in hsCRP from baseline as the primary endpoint, with data anticipated in YE.
 - **LLY:** In March, LLY announced the completion of the acquisition of Ventyx Biosciences, where the deal was announced post positive Ph2 data for oral, once-daily NLRP3 inhibitor VTX3232 in patients with obesity and CV risk

factors. VTX3232 demonstrated 78% reduction in hsCRP vs. 3% increase in placebo at week 12, with 69% patients achieving hsCRP normalization. Additionally, in combination with semaglutide, VTYX reported significant reduction in liver inflammation. Similar to BIOA, we expect a 15-20% 3-point MACE benefit to read-through favorably to VTYX's approach in CV risk.

- **NOVN:** Back in October 2025, Novartis closed the acquisition of Tourmaline Bio for \$1.4bn, which brought in pacibekitug, an IL-6 antibody with quarterly dosing intervals (which led to hsCRP reductions of 86%). We model \$2bn sales in 2037, which we include at a 40% PoS, following launch in 2031. If ZEUS is negative, leading to the market losing confidence in the IL-6 thesis, and moving to 0% PoS, implies a c.2% negative impact on our DCF. Conversely, a positive result, leading to greater confidence in the IL-6 thesis, and moving to a c.80% PoS, implies c.2% addition to our DCF.

Exhibit 3: Illustrative scenarios and potential stock moves for ZEUS

		NVO	BIOA
Scenario 1	Stat Sig / Meaningful	7%	+20-30%+
Scenario 2	No Stats but Signal	1%	-15%-20%
Scenario 3	Failure	-5%	-20-30%+

Source: Goldman Sachs Global Investment Research

Valuation and Risks

AMGN (Buy): Our 12-month PT of \$425 is based on a DCF analysis (2.5% TGR, 8% WACC). Downside risks: Commercial, clinical, regulatory risks: Market share of AMGN's established products erodes faster-than-expected as more biosimilar and generics are available; competitive landscape is crowded and other players are more successful, pipeline programs may be unsuccessful, failure to secure reimbursement could limit the commercial opportunity. Failure to protect IP portfolio. Failure to obtain regulatory approvals could lead to a lack of commercial success in the US and ex-US and pose significant downside risk to our sales estimates. Tax litigation risk.

IONS (Neutral): Valuation: Our 12-month price target of \$68 is based on our DCF analysis assuming an unchanged 13% WACC, 3% terminal growth rate, and risk adjusted DCF. Key risks: Upside risks: Clinical trial success with pipeline programs, regulatory approvals, better-than-expected product sales, new collaboration agreements could provide upside to our revenue and EPS estimates. Commercial risk - higher than anticipated sales and penetration into patient populations, earlier-than-expected approval timelines from the pipeline programs and competitive failures. Downside risks: Failure to receive regulatory and reimbursement approvals. Clinical and commercial setbacks. Lack of commercial success in the US and ex-US would pose significant downside risks to our sales estimates. Competitive risk: competitor assets could advance through clinical development ahead of expectations, generate data that appears superior, or achieve greater physician uptake to IONS' assets. Novel safety events could pose downside risks to our assumed penetration rates and timelines for regulatory approvals. Failure to protect IP portfolio from generic entry and financial risk, as IONS is not currently profitable.

We are Buy rated on AstraZeneca. Our 12-month price targets are derived from a 50:50 blend of DCF and P/E. Our bottom-up DCF analysis suggests a valuation of 15,722p per share per share (WACC 8.0%; TGR 2.5%), and on a multiples basis, we believe AstraZeneca should trade at 20x 2027E EPS, giving a valuation of 16,986p per share. As a result, our price targets are 16,370p/\$217 per share. Key risks to our view and price targets: (i) Clinical trial failure; (ii) Commercial execution; (iii) Pricing; (iv) Competitive developments; and (v) Patent exposure.

We are Neutral rated on Novo Nordisk. We derive our price target from a 50:50 blend of DCF and P/E approaches. Our bottom-up DCF analysis suggests a valuation of DKK 309 per share. We use a WACC of 7.1% and a TGR of -2%. On a multiples basis, we believe Novo Nordisk should trade on 14.5x P/E on our 2027 EPS estimate, implying DKK 310 per share. As a result, our 12-month price target is DKK 310. Our ADR PT is set with reference to the Danish line, translated at the current FX rate, leading to an ADR 12-month PT of \$48. Key risks to our view and price target include: Upside risks - (1) faster-than-expected Wegovy pill sales ramp post launch, and (2) higher-than-expected peak CagriSema sales. Downside risks - (1) clinical risks if CagriSema and/or amycretin development were to be unsuccessful; (2) slower-than-expected scale-up in manufacturing for Wegovy/Ozempic; (3) stronger-than-anticipated competitor obesity data, particularly oral small molecule GLP-1 based assets; and (4) deeper and sustained price pressure.

We are Sell rated on Novartis. Our 12-month price targets are derived from a 50:50

blend of DCF and P/E. Our bottom-up DCF analysis suggests a valuation of CHF 114 per share (WACC 7.1%, TGR 1%). On a multiples basis, we believe Novartis should trade on 14x 2027E EPS, giving a valuation of CHF 107 per share. As a result, our 12-month price targets are CHF 111/\$140 per ADR, which is c.13% downside to current levels. Key upside risks include: (i) ongoing positive earnings momentum; (ii) stronger-than-anticipated sales from key franchises; (iii) unforeseen/unexpected pipeline success; and (iv) capital allocation priorities, incl. BD and share buybacks.

NAMS: Valuation: We arrive at our 12-month target of \$38 using a discounted cash flow (DCF) analysis and M&A valuation at 85%/15% weightings, respectively. Our DCF valuation of \$38 is based on a WACC of 13% and TGR of 0%. Our M&A theoretical valuation of \$40 is based on 11.0x our modeled 2029 sales. **Downside risks:** Clinical: In the PREVAIL Phase 3 CVOT (cardiovascular outcomes trial), for which we expect data in 4Q26, if obicetrapib's MACE risk reduction is not statistically significant, or if the level of risk reduction is below that of competing therapies, obicetrapib could be less competitive, or face regulatory challenges. Additionally, if a safety concern emerges from the trial, obicetrapib's potential risks might be seen to outweigh its potential benefit. **Competitive Readthrough:** We expect obicetrapib to compete with oral PCSK9 inhibitors, the most advanced being MRK's MK-0616; if MK-0616 Phase 3 data readout in late 2025 yields a strong profile, obicetrapib could be less competitive. **Clinical Readthrough:** Obicetrapib has shown to lower Lp(a), but the benefit of this is unclear. If NVS' pelacarsen HORIZON study does not show MACE risk reduction, this would suggest broadly that Lp(a) lowering does not contribute meaningfully to risk reduction, and likely reduce the competitiveness of obicetrapib's profile. **Pricing/Access:** If approved, obicetrapib could experience pricing and/or access pressure, particularly with regard to government payors such as Medicare. Additionally, there could be pricing and competition risk near term from biosimilars, as Repatha's key patents expire in the 2029-2030 timeframe. **Commercial:** If obicetrapib is approved, NAMS will be competing against larger established pharmaceutical companies. If NAMS were to execute inefficiently, it would likely not be able to compete effectively. **Upside risks:** Clinical: If the PREVAIL Phase 3 CVOT achieves above/ on the higher end of MACE risk reduction expectations (above 20%), obicetrapib's competitive profile would strengthen. **Competition Setbacks:** If obicetrapib's competitors experience setbacks—notably if MRK's MK-0616 were to experience a trial readout delay, or yield a less favorable profile—obicetrapib would benefit competitively. **Clinical Readthrough:** If NVS' pelacarsen study indicates that lowering Lp(a) meaningfully contributes to MACE risk reduction, obicetrapib would have a more competitive profile given its observed impact on Lp(a). **Additional Validation/ Indication Expansion:** If obicetrapib produces meaningfully stronger data from additional trials, its competitive profile would strengthen. If new indications, such as Alzheimer's, are de-risked, this could provide upside. **Business Development/ M&A:** If NAMS were to establish a significant de-risking partnership or be acquired, its value could inflect significantly.

CRSP: Valuation: Our 12-month price target of \$48 is based on a 100% DCF value based on a WACC of 18% and TGR of 4%. **Risks:** Upside risks: earlier-than-expected timelines from the pipeline programs; other pipeline success and competitive failures. Downside risks: clinical setbacks; failure to generate durable clinical effect with allogeneic CAR Ts vs. competitors; high rate of non-specific edits/safety concerns; physician/payer pushback on use and coverage; regulatory headwinds; IP risk; manufacturing difficulties and financing/dilution.

Disclosure Appendix

Reg AC

We, Salveen Richter, CFA, James Quigley, Asad Haider, CFA, Tommie Reerink, CFA, Shyam Kotadia, Matt Dellatorre, Ph.D., Mark Aleynick, Ph.D., Shrunatra Mishra, Nick Jennings, Jeff Su and Richard Law, Ph.D., hereby certify that all of the views expressed in this report accurately reflect our personal views about the subject company or companies and its or their securities. We also certify that no part of our compensation was, is or will be, directly or indirectly, related to the specific recommendations or views expressed in this report.

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GS Factor Profile

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Growth is based on a stock's forward-looking sales growth, EBITDA growth and EPS growth (for financial stocks, only EPS and sales growth), with a higher percentile indicating a higher growth company. **Financial Returns** is based on a stock's forward-looking ROE, ROCE and CROCI (for financial stocks, only ROE), with a higher percentile indicating a company with higher financial returns. **Multiple** is based on a stock's forward-looking P/E, P/B, price/dividend (P/D), EV/EBITDA, EV/FCF and EV/Debt Adjusted Cash Flow (DACF) (for financial stocks, only P/E, P/B and P/D), with a higher percentile indicating a stock trading at a higher multiple. The **Integrated** percentile is calculated as the average of the Growth percentile, Financial Returns percentile and (100% - Multiple percentile).

Financial Returns and Multiple use the Goldman Sachs analyst forecasts at the fiscal year-end at least three quarters in the future. Growth uses inputs for the fiscal year at least seven quarters in the future compared with the year at least three quarters in the future (on a per-share basis for all metrics).

For a more detailed description of how we calculate the GS Factor Profile, please contact your GS representative.

M&A Rank

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Quantum

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The rating(s) for AstraZeneca, Novartis, Novartis (ADR), Novo Nordisk and Novo Nordisk (ADR) is/are relative to the other companies in its/their coverage universe: AstraZeneca, Bayer AG, GSK Plc, GSK Plc (ADR), Galderma, Lonza Group, Novartis, Novartis (ADR), Novo Nordisk, Novo Nordisk (ADR), Roche, Sandoz Group AG, Sanofi, Sanofi (ADR), Sartorius AG, Sartorius Stedim Biotech

The rating(s) for Amgen Inc., CRISPR Therapeutics and Ionis Pharmaceuticals Inc. is/are relative to the other companies in its/their coverage universe: 4D Molecular Therapeutics, Acadia Pharmaceuticals Inc., Agios Pharmaceuticals Inc., Alnylam Pharmaceuticals Inc., Amgen Inc., Annexon Biosciences, BioMarin Pharmaceutical Inc., Biogen Inc., CRISPR Therapeutics, Denali Therapeutics Inc., Enliven Therapeutics Inc., Generate Biomedicines Inc., Gilead Sciences Inc., Immunome Inc., Incyte Corp., Intellia Therapeutics, Ionis Pharmaceuticals Inc., Moderna Inc., Rapport Therapeutics, Regeneron Pharmaceuticals Inc., Relay Therapeutics, Sarepta Therapeutics Inc., Summit Therapeutics Inc., Taysha Gene Therapies Inc., Ultragenyx Pharmaceutical, Vertex Pharmaceuticals Inc., uniQure

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Goldman Sachs Investment Research global Equity coverage universe

	Rating Distribution				Investment Banking Relationships		
	Buy	Hold	Sell		Buy	Hold	Sell
Global	50%	34%	16%		65%	59%	43%

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Price target and rating history chart(s)

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Target price history table(s)

Amgen Inc. (AMGN)

Date of report	Target price (\$)	Closing price (\$)
01-May-26	425.00	329.82
04-Feb-26	415.00	366.20
08-Jan-26	403.00	330.11
05-Nov-25	400.00	319.86
06-Aug-25	405.00	284.67
14-Apr-25	400.00	293.92
08-Apr-25	408.00	280.26
08-Jan-25	370.00	264.21
31-Oct-24	375.00	320.16
04-Oct-24	369.00	319.66
07-Aug-24	370.00	312.50
03-May-24	350.00	311.29
15-Apr-24	359.00	265.51
15-Mar-24	361.00	268.87
07-Feb-24	350.00	295.87
22-Jan-24	313.00	310.16
01-Nov-23	300.00	260.84
13-Oct-23	305.00	284.10
04-Aug-23	310.00	243.28

Novo Nordisk (NOVOB.CO)

Date of report	Target price (Dkr)	Closing price (Dkr)
25-Jun-26	310.00	315.65
03-Jun-26	305.00	271.80
31-Mar-26	263.00	230.90
02-Mar-26	260.00	237.35
05-Feb-26	400.00	280.65
22-Jan-26	436.00	396.70
08-Dec-25	352.00	299.00
26-Nov-25	353.00	314.20
06-Nov-25	391.00	306.95
07-Aug-25	392.00	308.85
04-Aug-25	400.00	313.00
30-Jun-25	745.00	439.60
12-May-25	775.00	441.40
08-Apr-25	825.00	434.05
02-Apr-25	822.00	466.60
10-Feb-25	890.00	615.40
10-Jan-25	875.00	632.30
11-Nov-24	1,025.00	773.90
08-Oct-24	1,040.00	797.10
08-Aug-24	1,070.00	864.00
19-Jul-24	1,090.00	906.10
26-Jun-24	1,070.00	1,007.00
30-May-24	1,075.00	916.90

AstraZeneca (AZN.L)

Date of report	Target price (£)	Closing price (£)
01-Jul-26	16,370	13,854
30-Apr-26	16,525	13,948
02-Apr-26	16,600	15,316
11-Feb-26	16,496	14,818
27-Jan-26	16,389	13,880
07-Nov-25	15,504	12,900
02-Oct-25	15,163	12,454
16-Sep-25	15,013	11,346
30-Jul-25	14,883	11,498
02-Jul-25	14,806	10,390
28-May-25	14,524	10,386
30-Apr-25	14,482	10,728
08-Apr-25	15,318	10,374
28-Mar-25	15,130	11,416
10-Feb-25	15,067	11,718
10-Jan-25	15,558	10,982
13-Nov-24	15,955	10,144
08-Oct-24	16,046	11,678
05-Sep-24	15,914	12,540
26-Jul-24	15,756	12,180
04-Jul-24	15,131	12,106
30-May-24	15,200	12,028

NewAmsterdam Pharma (NAMS)

Date of report	Target price (\$)	Closing price (\$)
19-Feb-26	38.00	34.63
01-Dec-25	37.00	39.61
17-Oct-25	30.00	36.40
17-Jul-25	27.00	21.98

CRISPR Therapeutics (CRSP)

Date of report	Target price (\$)	Closing price (\$)
14-Jul-25	48.00	56.42
07-May-25	47.00	34.25
14-Apr-25	53.00	38.81
13-Feb-25	57.00	43.49
27-Jan-25	66.00	41.85
06-Nov-24	65.00	50.72
09-May-24	68.00	53.55
21-Feb-24	70.00	82.41
10-Dec-23	68.00	64.54
07-Nov-23	66.00	58.58
09-Aug-23	61.00	50.95

AstraZeneca (AZN)

Date of report	Target price (\$)	Closing price (\$)
01-Jul-26	217.00	183.86
30-Apr-26	219.00	187.37
02-Apr-26	220.00	203.49

Novo Nordisk (ADR) (NVO)

Date of report	Target price (\$)	Closing price (\$)
25-Jun-26	48.00	47.64
03-Jun-26	47.00	42.00
02-Mar-26	41.00	37.76
05-Feb-26	63.00	43.34
22-Jan-26	68.00	62.23
26-Nov-25	54.00	48.71
06-Nov-25	60.00	46.51
30-Sep-25	62.00	55.49
07-Aug-25	61.00	48.76
04-Aug-25	60.00	48.81
30-Jun-25	112.00	69.02
12-May-25	117.00	67.74
02-Apr-25	119.00	68.24
10-Jan-25	124.00	86.26
11-Nov-24	148.00	109.09
08-Oct-24	153.00	117.20
08-Aug-24	157.00	128.17
19-Jul-24	160.00	131.54
26-Jun-24	153.00	143.67
30-May-24	156.00	132.80

Ionis Pharmaceuticals Inc. (IONS)

Date of report	Target price (\$)	Closing price (\$)
09-Jul-26	68.00	64.27
24-Jun-26	75.00	76.52
29-Apr-26	74.00	74.83
25-Feb-26	72.00	80.93
29-Oct-25	69.00	71.88
26-Sep-25	65.00	64.09
02-Sep-25	45.00	57.49
30-Jul-25	40.00	43.37
02-Aug-24	37.00	49.13
07-May-24	33.00	40.93
21-Feb-24	29.00	43.53
02-Nov-23	28.00	43.73
10-Aug-23	25.00	39.82

Novartis (ADR) (NVS)

Date of report	Target price (\$)	Closing price (\$)
29-Jun-26	140.00	157.59
29-Apr-26	134.00	142.94
31-Mar-26	131.00	152.75
21-Jan-26	130.00	145.11
29-Oct-25	120.00	121.80
12-Sep-25	118.00	124.12
01-Jul-25	119.00	123.10
30-Apr-25	115.00	113.49
02-Apr-25	116.00	110.77
06-Feb-25	108.00	106.98
09-Jan-25	112.00	100.22
30-Oct-24	115.00	109.91
02-Oct-24	123.00	114.47
05-Sep-24	121.00	116.49
01-Jul-24	119.00	106.54
30-May-24	120.00	100.70

Novartis (NOVN.S)

Date of report	Target price (SFr)	Closing price (SFr)
29-Jun-26	111.00	126.20
29-Apr-26	106.00	113.34
31-Mar-26	105.00	120.86
06-Feb-26	101.00	119.96
21-Jan-26	104.00	114.34
29-Oct-25	95.00	98.50
12-Sep-25	94.00	98.85
18-Jul-25	95.00	92.29
01-Jul-25	97.00	97.90
30-Apr-25	95.00	93.79
02-Apr-25	102.00	96.82
06-Feb-25	98.00	97.71
30-Oct-24	100.00	95.35
02-Oct-24	104.00	97.23
05-Sep-24	103.00	98.91
19-Jul-24	105.00	94.20
01-Jul-24	106.00	96.72
30-May-24	109.00	91.23

Price targets shown in table(s) are unadjusted for corporate actions.

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